

INITIAL MEDICAL REPORT

Detailed fictional clinical report for testing and app demonstration

Hospital Name	SaveCare Multi-Speciality Hospital	Report Type	Initial Assessment Report
Patient Name	Ravi Kumar	Patient ID	RVK10245
Age / Sex	46 / Male	Date	20 March 2026
Department	General Medicine / Cardiology / Endocrinology	Consultant	Dr. A. Narayanan

Chief Complaints

- Severe fatigue and generalized body weakness
- Chest discomfort during walking or stair climbing
- Shortness of breath on mild exertion
- Frequent urination with increased thirst
- Poor sleep and reduced appetite
- Swelling in both feet with intermittent dizziness
- Blurred vision and reduced exercise tolerance

Vital Signs

Clinical Domain	Summary
Blood Pressure	168/104 mmHg
Heart Rate	112 bpm
Respiratory Rate	24 / min
Temperature	100.4 °F
SpO2	93%
Weight	92 kg
Height	168 cm
BMI	32.6 kg/m²

Detailed Clinical Problems and Values

No.	Clinical Problem / Finding	Observed Value	Status
1	Blood Pressure	168/104 mmHg	Severely elevated
2	Heart Rate	112 bpm	Tachycardia

3	Temperature	100.4 °F	Mild fever
4	Oxygen Saturation	93%	Low
5	Body Mass Index	32.6 kg/m²	Obese
6	Fasting Blood Sugar	186 mg/dL	High
7	Postprandial Blood Sugar	278 mg/dL	High
8	HbA1c	9.1%	Poor diabetic control
9	Total Cholesterol	262 mg/dL	High
10	LDL Cholesterol	176 mg/dL	High
11	Triglycerides	248 mg/dL	High
12	HDL Cholesterol	34 mg/dL	Low
13	Serum Creatinine	1.8 mg/dL	Elevated
14	Blood Urea	52 mg/dL	Elevated
15	Hemoglobin	10.2 g/dL	Mild anemia
16	Total WBC Count	13,900 /µL	Elevated
17	ALT (SGPT)	78 U/L	Elevated
18	AST (SGOT)	69 U/L	Elevated
19	Urine Protein	Positive	Abnormal
20	Urine Sugar	Positive	Abnormal

Clinical Summary

Clinical Domain	Summary
Metabolic	Diabetes remains poorly controlled with markedly elevated fasting sugar, postprandial sugar, and HbA1c.
Cardiovascular	Uncontrolled hypertension, tachycardia, chest discomfort, and ECG strain suggest high cardiovascular risk.
Renal	Raised serum creatinine, blood urea, and positive urine protein indicate renal stress.
Liver	ALT and AST are above normal, suggesting hepatic stress requiring monitoring.
Hematology	Low hemoglobin with elevated WBC count suggests mild anemia with possible inflammatory process.

Impression

The patient presents with multiple uncontrolled systemic abnormalities including severe hypertension, poorly controlled diabetes mellitus, dyslipidemia, obesity, renal stress, mild anemia, elevated liver enzymes, and probable inflammatory activity. Immediate treatment compliance, close monitoring, and multidisciplinary review are recommended.

Risk Category

CRITICAL / HIGH RISK

Recommended Care Plan

- Strict blood pressure control with regular daily monitoring
- Tight diabetic management with medication adherence and glucose charting
- Low-salt, low-sugar, low-fat diet with controlled calorie intake
- Renal function reassessment and urine monitoring
- Cardiology review for chest discomfort and ECG strain
- Hydration monitoring and swelling assessment
- Repeat complete laboratory evaluation within 2 to 4 weeks